



Understanding Sports Injuries Requiring Arthroscopic Surgery

ACL Reconstruction & LET · Meniscus
Repair · Rotator Cuff Repair · Frozen
Shoulder · Reverse Shoulder Replacement
· Shoulder Instability · Bankart Repair &
Remplissage · SLAP Tears

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BEFORE YOU BEGIN

What this guide covers

From how arthroscopic surgery works, through ACL, meniscus, rotator cuff and shoulder instability — to the rehabilitation phases that carry you back to sport.

PART 1

What Is Arthroscopic Surgery?

PART 2 — KNEE

The ACL Tear & When Surgery Is Needed · ACL Reconstruction — Graft Choices · ACL + Lateral Extra-Articular Tenodesis · Meniscus Tears & Repair · Knee Physiotherapy & Recovery Phases · Knee Arthroscopy FAQ

PART 3 — SHOULDER

The Rotator Cuff & Why Tears Progress · Shoulder Instability After Dislocation · Bankart Repair & Remplissage · SLAP Tears · Shoulder Physiotherapy & Recovery Phases · Shoulder Arthroscopy FAQ

PART 4

Preparing for Arthroscopic Surgery — General FAQ

The rehabilitation timelines in this booklet reflect the general phase-based structure used in well-established, criterion-based sports medicine physiotherapy programmes. Always follow the specific protocol given by your own team.

**ABOUT YOUR SURGEON****Dr. Harshil Shah**

Consulting Orthopaedic Surgeon specialising in joint replacement, arthroscopy and trauma. This booklet is intended purely for patient education — it does not replace a consultation, examination or individualised advice from your own surgeon and physiotherapist.

M.S. ORTHOPAEDICS**FIJR****FIAS**

1 PART 1

What is arthroscopic surgery?

A minimally invasive surgical technique in which a surgeon examines and treats joint problems through a few small (usually 5–10 mm) incisions called portals, rather than one large open incision. A thin camera — the arthroscope — projects a magnified, high-definition view onto a screen, while specialised instruments are passed through the other portals to repair or reconstruct damaged structures.



BECAUSE SURROUNDING TISSUE IS LARGELY UNDISTURBED, IT GENERALLY OFFERS

- ✓ Smaller scars and less soft-tissue trauma than open surgery
- ✓ Less post-operative pain and lower infection risk
- ✓ Shorter hospital stay — many done as day-care or overnight-stay surgery
- ✓ Faster initial recovery and an earlier start to structured physiotherapy
- ✓ The ability to directly visualise and treat multiple structures — ligament, meniscus/labrum, cartilage — in the same sitting

Both the knee and the shoulder are extremely well suited to arthroscopic treatment. ACL tears, meniscus tears, rotator cuff tears and shoulder instability are now treated arthroscopically as the standard of care in most centres, reserving open surgery for select complex or revision situations.

IN THE OPERATING ROOM

01

Small portals

A few 5–10mm incisions replace one large open cut.

02

Camera view

The arthroscope projects a magnified HD view to a screen.

03

Precise repair

Fine instruments repair or reconstruct through the other portals.

2 PART 2 — KNEE ARTHROSCOPY

Understanding the ACL tear

The anterior cruciate ligament (ACL) connects the thigh bone (femur) to the shin bone (tibia), preventing the tibia from sliding forward and controlling the knee's rotational stability — which is why it matters so much in sports involving sudden deceleration, pivoting, cutting and jumping.

Most ACL tears occur through a non-contact mechanism: landing awkwardly from a jump, a sudden change of direction, or a pivot with the foot planted, often combined with the knee buckling inward.

COMMON SYMPTOMS

- ⚠ A pop or snap, followed by rapid swelling (haemarthrosis)
- ⚠ A sense of instability, or the knee "giving way," especially on twisting
- ⚠ Pain along the joint line if the meniscus is also injured
- ⚠ Difficulty fully straightening or bending the knee due to swelling

DOES EVERY TEAR NEED SURGERY?

Not always. A minority of low-demand, typically older, sedentary patients with a stable knee and no desire to return to pivoting sport can be managed with structured physiotherapy alone.

Surgery is generally recommended for young, active patients, athletes returning to pivoting or contact sport, an unstable knee, or an associated repairable meniscus injury.

SPORTS WHERE ACL TEARS ARE MOST COMMON

Football / soccer

Basketball

Skiing

Handball

Badminton & racquet sports

Any sport with cutting or pivoting

2 KNEE — CONTINUED

ACL reconstruction — which graft is used?

A torn ACL can't simply be stitched — it's reconstructed using a strong tendon graft, threaded through drilled tunnels and fixed in place, gradually maturing into a new ligament over 9–12 months.

Hamstring tendon (autograft)

Semitendinosus ± gracilis from the patient's own inner thigh, folded into a multi-strand graft.

- + Smaller incision; less anterior knee/kneeling pain than BTB; very widely used.
- Some early hamstring weakness; slightly higher revision risk in young, high-pivoting athletes.

Bone-patellar tendon-bone / BTB (autograft)

Central third of the patellar tendon, harvested with a small bone plug at each end.

- + Bone-to-bone healing is fast and secure; long track record; low stretch-out over time.
- More anterior knee pain and pain with kneeling; small risk of patellar tendon weakening or fracture.

Quadriceps tendon (autograft)

Taken from above the kneecap, with or without a bone plug.

- + Thick, strong graft; growing evidence of low donor-site morbidity, outcomes comparable to BTB.
- Slightly less long-term data than BTB/hamstring; some early quadriceps weakness.

Allograft (donor tissue)

Processed tendon tissue from a tissue bank — no harvest from the patient's own body.

- + No donor-site pain; shorter surgical time.
- Higher failure/re-tear rate in young, active patients; generally for older, lower-demand patients.

The "best" graft is the one best suited to the individual patient — their age, sport, occupation (e.g., a lot of kneeling), revision status, and surgeon experience all factor into this shared decision.

2 KNEE — CONTINUED

ACL + lateral extra-articular tenodesis (LET)

LET is an additional stabilising procedure performed alongside the main ACL graft. A narrow strip of the iliotibial band is partially detached, re-routed around a point near the outer femur, and fixed under tension — creating an extra "checkrein" that resists the rotational, pivoting instability an ACL graft alone doesn't always fully control.

-38%

relative graft failure risk — in the STABILITY trial, adding LET at the time of primary surgery reduced clinically significant graft failure by roughly 38%, and cut outright graft rupture by roughly two-thirds, versus an isolated ACL reconstruction in this same high-risk group.

RECOMMENDED WHEN A PATIENT MEETS TWO OR MORE OF THESE HIGH-RISK CRITERIA



Age under ~25, especially returning to competitive pivoting or contact sport



A high-grade pivot-shift (grade 2+) found on examination under anaesthesia



Generalised ligamentous laxity or knee hyperextension beyond 10°



Revision ACL surgery, or MRI/X-ray risk markers (steep tibial slope, deep notch, Second fracture)

Because a failed graft usually means a more complex revision, a longer recovery and generally inferior outcomes, addressing known risk factors up front with LET — in patients who genuinely qualify — is the more durable path. It is not a routine "add-on" for every ACL patient; it is targeted specifically at the higher-risk group above.



In practice: LET adds a modest amount of operating time and one small incision on the outer thigh — it doesn't typically change overall hospital stay or early rehab timeline much.

2 KNEE — CONTINUED

Meniscus tears

Each knee has two C-shaped cartilage cushions — the medial (inner) and lateral (outer) meniscus — that absorb shock, distribute load, and add secondary stability. Tears are extremely common alongside ACL injuries, and also occur in isolation from twisting injuries or degenerative wear.

YOUNG, ACTIVE PATIENTS

Longitudinal / bucket-handle tear

A long tear running along the length of the meniscus; a large inner fragment can flip into the centre of the joint like the handle of a bucket, physically blocking the knee from fully straightening (a "locked knee").

MECHANICALLY DIFFICULT TO REPAIR

Radial tear

A tear running across the fibres of the meniscus, perpendicular to its usual circular pattern — this disrupts the meniscus's ability to spread load.

OFTEN DEGENERATIVE

Flap / horizontal cleavage tear

Splits the meniscus horizontally or creates an unstable flap; more common in middle-aged and older patients.

EFFECTIVELY UNLOADS THE MENISCUS

Root tear

A tear at the very back (posterior root) attachment — biomechanically similar in impact to having no meniscus at all if left untreated.



A "locked knee" — when a bucket-handle fragment blocks full extension, this is often treated more urgently than a routine tear.

WHETHER A TEAR CAN BE REPAIRED DEPENDS ON



The tear pattern itself



Which vascular zone it's in



Patient age & tissue quality

2 KNEE — CONTINUED

The "zone" concept — why some tears heal

The meniscus is divided into vascular zones that determine healing potential.

Outer (near capsule)

Red-red zone — good blood supply

The outer 10–25%, close to the capsule — repairs here heal reliably.

Middle (border)

Red-white zone — partial blood supply

The middle border zone — repairs here can heal well, especially in younger patients.

Inner (central)

White-white zone — no blood supply

The inner two-thirds — tears here generally cannot heal even if stitched, usually treated with partial meniscectomy instead.

REPAIR VS. PARTIAL MENISCECTOMY

Whenever healing is realistic — typically longitudinal and bucket-handle tears in the red-red or red-white zones, especially under about 40 — repair is strongly preferred over trimming, because removing meniscal tissue raises contact pressure through the joint and meaningfully increases the long-term risk of early osteoarthritis. Repair is especially favoured when done alongside an ACL reconstruction, which biologically improves meniscal healing rates.

In older patients, or when the tear lies in the avascular white-white zone, partial meniscectomy remains the more practical and reliable option.



Preserving meniscal tissue whenever it's safe to do so protects the joint's long-term health — this is why repair is favoured wherever healing is realistic.

2 KNEE — CONTINUED

Physiotherapy for the knee — prehabilitation

Whether or not surgery is eventually needed, a period of "prehabilitation" — or a full non-operative programme for patients choosing not to have surgery — generally focuses on:

Reduce swelling, restore motion

Restoring full passive knee extension and flexion before surgery — operating on a stiff, swollen knee raises stiffness risk afterward.

Quadriceps activation

Activation and strengthening — often the single strongest predictor of a smooth early recovery.

Hip, hamstring & gait work

Hamstring and hip/gluteal strengthening, gait retraining and single-leg balance work.

For non-operative patients

Bracing during pivoting activity plus structured neuromuscular training, with periodic reassessment.

Most academic sports medicine programmes structure ACL and meniscus rehabilitation into criterion-based phases — a patient progresses when they meet specific strength, swelling and range-of-motion milestones, not simply by the calendar date.

Calendar-based

"It's been 6 weeks, so..."

Criterion-based

"Strength and motion milestones are met"

2 KNEE — CONTINUED

Post-operative recovery — the phase framework

01 0-2 WEEKS — PROTECTION

Control & activate

Control swelling and pain, restore full passive knee extension early, activate the quadriceps (often with electrical muscle stimulation), begin gentle range of motion, weight-bear as advised.

02 2-6 WEEKS — EARLY STRENGTHENING

Off crutches

Normalise walking pattern off crutches, progress closed-chain strengthening (mini-squats, leg press), restore full range of motion, begin stationary cycling.

03 6-12 WEEKS — PROGRESSIVE STRENGTHENING

Build symmetry

Build quadriceps/hamstring strength symmetry, introduce balance and proprioceptive training, begin controlled multi-plane movements.

04 3-5 MONTHS — NEUROMUSCULAR CONTROL & RUNNING

Structured running

Begin a structured running progression once strength criteria are met, add plyometrics and agility drills, sport-specific movement patterns.

05 6-9+ MONTHS — RETURN TO SPORT

Formal testing before clearance

Formal return-to-sport testing (strength symmetry, hop tests, movement-quality screening) before clearance for pivoting/contact sport; returning before 8-9 months carries a meaningfully higher re-injury rate in young athletes.

2 KNEE ARTHROSCOPY FAQ

The questions patients ask most

How long does ACL surgery take?

A standard arthroscopic ACL reconstruction typically takes about 1–1.5 hours; adding a meniscus repair or LET adds some additional time.

When can I walk after ACL surgery?

Most can walk with crutches the same or next day, progressing to full weight-bearing over 1–3 weeks depending on whether a meniscus repair was also done.

Can I go back to running?

Straight-line running usually begins around 3–4 months, once strength criteria are met; cutting/pivoting sport is introduced later, closer to 6–9 months.

Is meniscus repair always possible?

No — it depends on the tear pattern, blood supply zone, tissue quality and chronicity. Tears in the avascular white-white zone usually cannot be repaired.

Will I need to stay in hospital?

Most patients go home the same day or after one overnight stay, depending on the surgeon's practice and anaesthesia used.

How soon can I drive again?

Typically once you have good quadriceps control and are off strong pain medication — often 3–6 weeks for a right-knee reconstruction.

What if a meniscus tear is left untreated?

An unstable fragment can continue to cause locking, catching and pain, and over years untreated damage significantly raises the risk of early osteoarthritis.

What's the risk of re-tearing the graft?

Reported failure rates vary with age, sport and risk factors, but are meaningfully reduced in high-risk young athletes when LET is added, and further reduced by full criterion-based rehab.

Also worth asking — why your surgeon recommends a specific graft, whether you'll need a brace, and whether both knees could ever be treated at once. These depend on your individual anatomy and are worth raising directly.

3 PART 3 — SHOULDER ARTHROSCOPY

The rotator cuff — why it matters

The rotator cuff is a group of four muscles and tendons — supraspinatus, infraspinatus, teres minor, subscapularis — that wrap around the humeral head and hold it centred within the shallow shoulder socket, while allowing an enormous range of motion.



WHY TEARS HAPPEN

- ⚠️ Acute traumatic tears — a fall onto an outstretched arm, a heavy lifting injury, or a dislocation, especially over 40
- 🕒 Degenerative/attritional tears — gradual wear from age, repetitive overhead loading, reduced tendon blood supply

TYPICAL SYMPTOMS

- ⚠️ Pain with overhead activity, and night pain lying on the affected side
- ⚠️ Weakness with overhead lifting or external rotation; difficulty reaching behind the back

Why a torn cuff often needs repairing — unlike many soft-tissue injuries, a torn rotator cuff tendon doesn't reliably heal itself — the torn end retracts and loses its blood supply. Left alone, a tear tends to enlarge, and the muscle can undergo fatty infiltration (fat replacing muscle fibres), a change that is essentially irreversible once advanced. Repairing earlier, while tendon quality and muscle bulk are still good, gives a much more reliable outcome.

THE FOUR ROTATOR CUFF MUSCLES

Supraspinatus

Infraspinatus

Teres minor

Subscapularis

3 SHOULDER — CONTINUED

How an untreated cuff injury can progress

01 THE SHOULDER STOPS MOVING NORMALLY

Pain, guarding & stiffness

Persistent pain leads patients to instinctively protect the arm. This guarding, combined with inflammation, can allow the shoulder capsule to gradually thicken and tighten.

02 BOTH ACTIVE AND PASSIVE MOTION RESTRICT

Secondary frozen shoulder

Capsular tightening produces a secondary adhesive capsulitis. Key distinction: in frozen shoulder, both active and passive movement are restricted, whereas in an isolated cuff tear, passive range is usually preserved. Diabetes, thyroid disease and prolonged immobilisation raise this risk.

03 YEARS OF AN UNTREATED MASSIVE TEAR

Cuff tear arthropathy

The humeral head loses the cuff's centred pull and migrates upward against the acromion, damaging cartilage and bone. Advanced cases develop pseudoparalysis — the deltoid is willing to fire, but without a functioning cuff, the patient cannot actively lift the arm above shoulder height.

04 WHEN THE CUFF CAN'T BE RESTORED

Reverse shoulder replacement

A specifically designed implant swaps the normal ball/socket anatomy — ball on the glenoid, socket on the humerus — letting the (almost always healthy) deltoid take over the cuff's job, restoring the ability to lift the arm.

3 SHOULDER — CONTINUED

When is reverse shoulder replacement indicated?

It is generally reserved for older patients (classically over roughly 65–70, though increasingly used in carefully selected younger patients with irreparable pathology) — the priority in this group is dependable pain relief and functional overhead motion for daily life, rather than restoring the shoulder for high-demand athletic use.

GENERALLY INDICATED FOR

- ✓ Established rotator cuff tear arthropathy (cuff tear plus significant joint arthritis)
- ✓ Failed previous rotator cuff repair with ongoing pain and dysfunction
- ✓ Massive, irreparable tears causing pseudoparalysis, particularly in older, lower-demand patients
- ✓ Selected proximal humerus fractures or fracture non-unions where cuff repair isn't realistic

GENERALLY NOT RECOMMENDED WHEN

A patient still has full active elevation despite a massive tear

The deltoid muscle itself is not functioning — the mechanism relies entirely on a working deltoid



Anatomic

Ball on humerus, relies on the cuff



Reverse

Ball on glenoid, deltoid does the work

3 SHOULDER — CONTINUED

Shoulder instability after dislocation

A traumatic shoulder dislocation — most often during a fall onto an outstretched or abducted, externally rotated arm in contact or overhead sports — commonly produces two characteristic injuries as the humeral head is forced out of the socket.

PRIMARY SOURCE OF INSTABILITY

Bankart lesion

A tear of the anteroinferior labrum away from the glenoid bone, often together with stretching of the capsular ligaments.

+

ON THE HUMERAL HEAD

Hill-Sachs lesion

A compression fracture/dent on the back-outer part of the humeral head, created as it impacts the front edge of the glenoid during dislocation.

Together, a "bipolar" lesion — whether a Hill-Sachs lesion is likely to "re-engage" the glenoid rim during movement (making it "off-track") depends on its size and position relative to glenoid bone loss. Surgeons assess this using the "glenoid track" concept on CT or MRI, and it materially changes the surgical plan.

RECURRENCE RISK RISES WITH

Younger age

Ongoing collision sport

Greater glenoid bone loss

3 SHOULDER — CONTINUED

Bankart repair & posterior capsular remplissage

THE STANDARD FIX

Arthroscopic Bankart repair

For most first-time or recurrent anterior dislocations with a repairable labrum and less than roughly 15–20% glenoid bone loss, the torn labrum and capsule are reattached to the front of the glenoid using small suture anchors, restoring the socket's depth and capsular tension.

WHEN ADDED

Adding remplissage

When imaging shows a significant, "off-track" Hill-Sachs lesion likely to catch on the glenoid rim, surgeons suture the posterior capsule and infraspinatus tendon into the defect — filling it and converting it from an inside, joint-engaging defect into an outside one.

Combining Bankart repair with remplissage has been shown to achieve recurrence rates approaching those of bone-grafting procedures (such as the Latarjet procedure), while remaining a soft-tissue, arthroscopic operation. It is generally preferred over Latarjet when glenoid bone loss remains below the "critical" threshold (roughly 20–25%), with Latarjet reserved for larger glenoid bone deficits or failed prior soft-tissue repairs.

Under 15–20%

bone loss — Bankart repair alone

Off-track

Hill-Sachs — add remplissage

Over 20–25%

bone loss — Latarjet considered



These thresholds guide the decision, but your surgeon weighs them alongside your age, sport and activity demands, and prior surgical history.

3 SHOULDER — CONTINUED

SLAP tears

A SLAP tear (Superior Labrum, Anterior to Posterior) injures the top part of the labrum, at or near the attachment of the long head of the biceps tendon — common in overhead athletes (throwers, swimmers, tennis and volleyball players) from repetitive loading, or from a fall onto an outstretched arm.

BASIC CLASSIFICATION

- | | |
|---|--|
| I Fraying of the labrum with an intact biceps anchor; often just trimmed | II Most common — labrum and biceps anchor detached, creating true instability |
| III Bucket-handle tear with an intact biceps anchor; unstable fragment trimmed | IV Bucket-handle tear extending into the biceps tendon itself |

Younger throwing athletes with a true Type II tear often do well with arthroscopic repair, while in older, less athletically demanding patients, biceps tenodesis — releasing and re-fixing the biceps anchor lower down — frequently gives more reliable pain relief with a simpler recovery than attempting to repair the labrum.



Type II is the most commonly diagnosed pattern, and the one where the repair-versus-tenodesis decision matters most.

3 SHOULDER — CONTINUED

Physiotherapy for the shoulder — conservative management

Not every rotator cuff tear needs surgery immediately. Small, partial-thickness, or chronic degenerative tears in lower-demand or older patients — particularly when active motion and strength remain reasonable — are often first managed with a structured non-operative programme built around:

Settle the initial pain

Activity and load modification to settle the initial pain.

Strengthen the cuff

Scapular stabiliser and rotator cuff strengthening exercises.

Retrain mechanics

Posture and shoulder-blade mechanics retraining.

Graded return

Gradual, graded return to overhead activity, with regular reassessment.

Frozen shoulder needs a different pace — the initial emphasis is on pain control (which may include an injection) combined with gentle, consistent stretching through the painful and stiffening phases, progressing to more assertive stretching as the shoulder moves into the "thawing" phase. Aggressive early stretching of a truly frozen shoulder can sometimes aggravate symptoms, so pacing is guided by which phase the shoulder is in.

3 SHOULDER — CONTINUED

Post-operative recovery — two rehab tracks

A repaired tendon or labrum needs time to biologically heal onto bone — recovery is deliberately conservative in its early phases, whichever procedure was performed.

Rotator cuff repair

- 01** WEEKS 0-6 · PROTECTION
Sling & passive motion only
Sling worn for support; only passive/assisted range of motion by the therapist — protecting the healing repair.
- 02** WEEKS 6-12 · ACTIVE-ASSISTED
Sling weaned, motion progresses
Progress from passive to active-assisted then active range; gentle scapular and light cuff activation begins.
- 03** ~3-4.5 MONTHS · STRENGTHENING
Resistance work begins
Resistance-band and light weighted strengthening for the cuff and scapular stabilisers.
- 04** ~4.5-6+ MONTHS · RETURN
Sport-specific & overhead work
Return to overhead sport or heavy manual work generally not cleared until criteria are met, often 5-6 months.

Bankart repair (± remplissage)

- 01** WEEKS 0-4 · PROTECTION
Sling worn continuously
Except for hygiene/exercises; external rotation and abduction beyond safe limits avoided. Gentle pendulum motion only.
- 02** WEEKS 4-6 · MOTION RESTORATION
Sling gradually weaned
Active-assisted motion progressed within protected limits, guarding against excess rotation early.
- 03** WEEKS 6-12 · STRENGTHENING
Restoring full function
Progressive rotator cuff and scapular strengthening, restoring full functional range of motion.
- 04** ~4-6+ MONTHS · RETURN TO SPORT
Sport-specific & contact progression
Return to collision or overhead sport generally not cleared before roughly 5-6 months.

When remplissage is added, external rotation is typically restricted a little more conservatively in the early weeks, since the procedure involves the infraspinatus tendon and posterior capsule directly.

3 SHOULDER ARTHROSCOPY FAQ

The questions patients ask most

Cuff tear or frozen shoulder — how do I know?

A useful clue is passive motion: in a cuff tear, someone else can usually still move your arm through a nearly full range; in frozen shoulder, even passive movement is restricted.

Why operate soon rather than wait?

Tears tend to enlarge and the muscle can undergo fatty degeneration over time — earlier repair generally gives more reliable healing.

Will every large tear need a reverse replacement?

No — many large and even massive tears remain repairable, especially in younger patients with good tissue quality.

Does a SLAP tear always need surgery?

No — many low-grade tears respond to physiotherapy, while higher-grade tears in younger athletes may need repair or biceps tenodesis.

Can a small cuff tear heal on its own?

Tendon tears don't reliably heal spontaneously because the torn edge retracts and loses blood supply, though small, low-grade partial tears can sometimes remain stable with physiotherapy.

Total vs. reverse replacement — the difference?

A standard (anatomic) replacement relies on an intact cuff; a reverse replacement lets the deltoid move the arm without a working cuff.

Will my shoulder dislocate again after Bankart repair?

A well-selected repair (with remplissage when indicated) substantially reduces recurrence risk, though no procedure eliminates it entirely.

When can I return to overhead sport or throwing?

Generally not before about 5–6 months, and only once formal strength and motion criteria are met — throwers follow a specific interval throwing programme.

Also worth asking — why a remplissage was added to your plan, how long you'll need a sling, and whether instability can ever be managed without surgery in your specific case.

4 PART 4 — PREPARING FOR SURGERY

General questions that apply across knee & shoulder alike

How should I prepare for surgery?

Follow your surgeon's pre-op instructions (fasting, medication adjustments), arrange help at home, prepare your recovery area, and complete any "prehab" exercises given.

How painful is recovery?

Best managed proactively with your medication schedule, ice/cryotherapy, elevation and early gentle movement; pain typically peaks in the first 2–4 days and steadily improves.

What are the general risks?

As with any surgery: infection, bleeding, blood clots, stiffness, nerve or vessel injury, and the possibility the repair doesn't heal as expected. Uncommon with modern technique.

Can I do rehab at home instead?

A combination is ideal: supervised sessions let your therapist assess progress and correct technique, while a home programme keeps you progressing between visits.

Will I be under general anaesthesia?

Most knee and shoulder arthroscopies use general anaesthesia, often combined with a regional nerve block — especially for shoulder surgery — to improve early pain control.

When can I return to work?

Desk-based work is often possible within 1–2 weeks for most knee procedures and slightly longer for shoulder; manual or overhead work typically needs several months.

Why is physiotherapy so important?

Surgery restores the anatomy, but physiotherapy restores the strength, motion and neuromuscular control needed to actually function well and resist re-injury.

How do I know I'm ready to return to sport?

Readiness is best judged with objective testing — strength symmetry, hop or functional tests — not by months passed or pain alone.

Bring this booklet to your consultation — use it to prepare sharper questions about your own anatomy, tear pattern and goals. Every recovery is individual.



Understand why a technique was chosen



Commit to the rehab phases, in order



Judge readiness by testing, not the calendar



A final word

Recovering well is a partnership between your surgeon, your physiotherapist — and you.

Understanding why a particular graft, technique, or additional procedure has been recommended — and committing to the structured rehabilitation phase that follows — gives you the best chance of a durable, high-functioning outcome and a safe return to the activities and sport you enjoy. Please use this booklet as a starting point for a conversation with your own treating team.

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